

Statutory citations in provider claim-denial appeal letters

From	Fred Ugast, U.S. Photovoltaics, Inc. (CivicScale) — fred.ugast@uspv.co
Subject	Review of twelve legal provisions proposed for citation in appeal letters
Jurisdiction	Ohio
Estimated effort	One to two hours. Twelve short sheets plus six general questions.
Status	Pre-launch. No letter containing these citations has been sent to any payer.

What we are asking you to do

CivicScale is building software that drafts appeal letters for medical practices whose insurance claims have been denied. Each letter argues the practice's case and, where useful, cites the legal provision that supports it.

We have built an automated check that verifies three things about every citation before a letter can use it: that the provision exists, that its actual text contains whatever we quote from it, and that it applies to the type of payer being written to. Nothing reaches a letter unless all three hold.

What our check cannot do is tell us whether a provision is the *right* one to cite for a given denial reason. That is a legal judgement, and it is the only thing standing between the current state — letters that cite no law at all — and letters that cite law we can stand behind.

Below are twelve provisions we have selected as candidates, one sheet each. For each, we ask you to tell us whether it does what we think it does and whether citing it is sound. Six broader questions follow.

What we are not asking

- We are not asking you to draft or review letter language beyond the single sentence shown on each sheet.
- We are not asking for an opinion on the software, the business, or anything outside these twelve provisions and the six questions.
- We are not asking you to research provisions we have not listed — though if an obvious better one comes to mind, we very much want to know.
- This is a request for review, not an engagement letter. Please scope and quote as you see fit.

Why this matters more than it might appear

An earlier version of this software generated its legal citations without checking them. Of nineteen provisions it cited across ten test letters, twelve did not say what the letter claimed. One example: the letters repeatedly cited Ohio Adm. Code 3901-1-54 as a health-claims payment standard. Its actual subject is unfair property and casualty claims settlement practices.

Those letters went to no one — they were generated in testing and the defect was found before launch. The automated check described above was built in response, and the software currently cites no statute whatsoever. We would rather cite nothing than cite wrongly, and we will keep citing nothing until someone qualified has signed off on what we cite.

How to respond

Mark up this document however suits you — tick boxes, write in the ruled space, use tracked changes or comments, or reply separately referencing the sheet numbers. Short answers are fine. Where a provision is wrong for our purpose, the most useful thing you can tell us is what would be right instead, or that nothing would be.

Each provision sheet asks four questions. The first two are the ones that gate the software: a provision only becomes usable if both are affirmative.

THE FOUR QUESTIONS ON EACH SHEET

- 1. Does this provision impose the obligation we describe?
 - 2. Is it appropriate to cite for this denial reason and this payer type?
 - 3. If not — is there a better provision, or none?
 - 4. Any wording you would require or forbid when it is cited?
-

Six general questions

These matter more to us than any single provision, because each one shapes how the software behaves across every letter it will ever write.

Q1. Should a first-level appeal to a commercial payer cite statute at all?

We genuinely do not know. We have heard both that statutory citation signals seriousness and gets claims reworked, and that it antagonises claims staff who have no authority to act on it and slows things down. If citation belongs only at second level or external review, we would rather build that in than discover it.

Q2. Does Medicare authority carry any weight with a commercial payer?

Our check currently refuses to let a letter cite Medicare-derived authority — the CMS claims processing manuals, the National Correct Coding Initiative — when the payer is a commercial insurer, on the reasoning that those bind Medicare contractors and not Anthem. But we understand many commercial payers adopt NCCI edits contractually in their provider agreements. If that makes our refusal too strict, we would loosen it — but only on advice, and we would want to know how to frame such a citation so it is accurate about what it is.

Q3. Are we missing the provisions that actually work?

Our twelve were assembled by reading the Ohio Revised Code and the CMS manuals directly. Someone who does this work will know which provisions practices actually cite to effect, and which are technically apposite but practically inert. Please tell us what we should be citing that is not on the list.

Q4. Does a software-generated letter citing law create any exposure for the practice sending it?

The letter goes out over the practice's name and signature. We want to understand whether anything about that arrangement — the citation of legal provisions in a document prepared by software, sent by a non-lawyer to an insurer in pursuit of payment — creates risk for the practice, for us, or for both. Unauthorised practice of law, misrepresentation, or anything else we have not thought of.

Q5. How strongly should the letter assert?

There is a difference between “Ohio law requires payment within thirty days” and “we understand Ohio law to require payment within thirty days.” We can write either, uniformly, across every letter. Which is right, and does it change by provision or by payer type?

Q6. Does this structure transfer to other states?

We are starting with Ohio. Our intention is that each additional state is a separate set of provisions reviewed the same way, and that until a state has been reviewed its letters cite nothing. Is that the right shape, or do the differences between states run deeper than a per-state citation list can capture?

The twelve provisions

One sheet each. Each sheet gives the denial reason, the payer type, the provision, its retrieved text, and the exact sentence a letter would build from it — then asks the four questions.

Denial reason codes are the standard CARC codes used on remittance advice; the plain meaning is given on each sheet.

Ohio Revised Code § 3901.381

Denial reason	CO-16 — Claim/service lacks information or has submission/billing error(s).
Payer type	Commercial (non-Medicare)
Official heading	Third-party payers processing claims for payment for health care services
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.381

RETRIEVED TEXT

... s adopted by the superintendent of insurance under section 3902.22 of the Revised Code, the third-party payer shall pay or deny the claim not later than thirty days after receipt of the claim. When a third-party payer denies a claim, the third-party payer shall notify the provider and the beneficiary. The notice shall state, with specificity, why the third-party payer denied the claim. (2)(a) Unless division (B)(3) of this section applies, when a provider or beneficiary has used the standard claim form, but the third-party payer determines that reasonable supporting documentation is needed to establish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than ...

... ish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than forty-five days after receipt of the claim. Supporting documentation includes the verification of employer and beneficiary coverage under a benefits contract, confirmation of premium payment, medical information regarding the beneficiary and the services provided, information on the responsibility of another third-party payer to make payment or confirmation of the amount of payment by another third-party payer, and information that is needed to correct material deficiencies in the claim related to a diagnosis or treatment or the provider's identification. Not later than thirty days after receipt of ...

... provided in the claim is materially deficient, the third-party payer shall notify the provider or beneficiary not later than fifteen days after receipt of the claim. The notice shall state, with specificity, the information needed to correct all material deficiencies. Once the material deficiencies are corrected, the third-party payer shall proceed in accordance with division (B)(1) or (2) of this section. It is not a violation of the notification time period of not more than fifteen days if a third-party payer fails to notify a provider or beneficiary of material deficiencies in the claim related to a diagnosis or treatment or the provider's identification. A third-party payer may request the information necessary to co ...

THE SENTENCE A LETTER WOULD WRITE

A third-party payer must notify the provider of a material deficiency not later than fifteen days after receipt of the claim, and pay or deny a clean claim not later than thirty days after receipt (forty-five days when supporting documentation is needed).

WHY WE SELECTED IT

CO-16 is a missing-information denial; 3901.381 is the Ohio section that sets the deficiency-notice clock and the pay-or-deny clock for a claim on the standard form.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.381

Denial reason	CO-45 — Charge exceeds fee schedule / maximum allowable or contracted or legislated fee arrangement.
Payer type	Commercial (non-Medicare)
Official heading	Third-party payers processing claims for payment for health care services
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.381

RETRIEVED TEXT

... s adopted by the superintendent of insurance under section 3902.22 of the Revised Code, the third-party payer shall pay or deny the claim not later than thirty days after receipt of the claim. When a third-party payer denies a claim, the third-party payer shall notify the provider and the beneficiary. The notice shall state, with specificity, why the third-party payer denied the claim. (2)(a) Unless division (B)(3) of this section applies, when a provider or beneficiary has used the standard claim form, but the third-party payer determines that reasonable supporting documentation is needed to establish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than ...

THE SENTENCE A LETTER WOULD WRITE

A third-party payer shall pay or deny a claim not later than thirty days after receipt.

WHY WE SELECTED IT

The deadline that governs reprocessing once the fee-schedule dispute is resolved; the contracted rate itself comes from the provider agreement, not from statute.

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1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.389

Denial reason	CO-45 — Charge exceeds fee schedule / maximum allowable or contracted or legislated fee arrangement.
Payer type	Commercial (non-Medicare)
Official heading	Computation of interest
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.389

RETRIEVED TEXT

(A) Any third-party payer that fails to comply with section 3901.381 of the Revised Code, or any contractual payment arrangement entered into under section 3901.383 of the Revised Code, shall pay interest in accordance with this section. (B) Interest shall be computed based upon the number of days that have elapsed between the date payment is due in accordance with section 3901.381 of the Revised Code or the contractual payment arrangement entered into under section 3901.383 of the Revised Code, and the date payment is made. The interest rate for determining the amount of interest due shall be equal to an annual perc ...

... d the date payment is made. The interest rate for determining the amount of interest due shall be equal to an annual percentage rate of eighteen per cent. (C) For purposes of this section, if a dispute exists between a provider and a third-party payer as to the day a payment was made by the third-party payer, both of the following apply: (1) If the third-party payer or a person acting on behalf of the third-party payer submits a payment directly to a provider by mail and retains a record of the day the payment was mailed, there exists a rebuttable presumption that the payment was made five business days before the day the payment was received by the provider, unless it can be proven otherwise. (2) If the third-party paye ...

THE SENTENCE A LETTER WOULD WRITE

Interest on a claim not paid when due accrues at an annual percentage rate of eighteen per cent and is paid directly to the provider.

WHY WE SELECTED IT

An underpayment left unpaid past the 3901.381 deadline carries statutory interest; this is the section that says how much.

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1. Does this provision impose the obligation we describe?

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2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.381

Denial reason	CO-97 — The benefit for this service is included in the payment or allowance for another service that has already been adjudicated.
Payer type	Commercial (non-Medicare)
Official heading	Third-party payers processing claims for payment for health care services
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.381

RETRIEVED TEXT

... s adopted by the superintendent of insurance under section 3902.22 of the Revised Code, the third-party payer shall pay or deny the claim not later than thirty days after receipt of the claim. When a third-party payer denies a claim, the third-party payer shall notify the provider and the beneficiary. The notice shall state, with specificity, why the third-party payer denied the claim. (2)(a) Unless division (B)(3) of this section applies, when a provider or beneficiary has used the standard claim form, but the third-party payer determines that reasonable supporting documentation is needed to establish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than ...

THE SENTENCE A LETTER WOULD WRITE

A third-party payer shall pay or deny a claim not later than thirty days after receipt.

WHY WE SELECTED IT

Ohio statute says nothing about bundling; the only Ohio obligation a CO-97 letter can state is the pay-or-deny clock on reprocessing. The coding argument rests on CPT conventions, which are not free primary law and stay generic.

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☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.381

Denial reason	CO-4 — The procedure code is inconsistent with the modifier used, or a required modifier is missing.
Payer type	Commercial (non-Medicare)
Official heading	Third-party payers processing claims for payment for health care services
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.381

RETRIEVED TEXT

... s adopted by the superintendent of insurance under section 3902.22 of the Revised Code, the third-party payer shall pay or deny the claim not later than thirty days after receipt of the claim. When a third-party payer denies a claim, the third-party payer shall notify the provider and the beneficiary. The notice shall state, with specificity, why the third-party payer denied the claim. (2)(a) Unless division (B)(3) of this section applies, when a provider or beneficiary has used the standard claim form, but the third-party payer determines that reasonable supporting documentation is needed to establish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than ...

THE SENTENCE A LETTER WOULD WRITE

A third-party payer shall pay or deny a claim not later than thirty days after receipt.

WHY WE SELECTED IT

As for CO-97: the modifier argument is CPT convention; the statute supplies only the clock.

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1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3922.01

Denial reason	CO-50 — Non-covered service because it is not deemed a medical necessity by the payer.
Payer type	Commercial (non-Medicare)
Official heading	Definitions
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3922.01

RETRIEVED TEXT

As used in this chapter: (A) "Adverse benefit determination" means a decision by a health plan issuer: (1) To deny, reduce, or terminate a requested health care service or payment in whole or in part, including all of the following: (a) A determination that the health care service does not meet the health plan issuer's requirements for medical necessity, appropriateness, health care setting, level of care, or effectiveness, including experimental or investigational treatments; (b) A determination of an individual's eligibility for individual health insurance coverage, including coverage offered to individuals through a nonemployer group, to ...

... ts to a covered person; (3) Payment for the provision of health care services to or for a covered person. (R) "Independent review organization" means an entity that is accredited to conduct independent external reviews of adverse benefit determinations pursuant to section 3922.13 of the Revised Code. (S) "Medical or scientific evidence" means evidence found in any of the following sources: (1) Peer-reviewed scientific studies published in, or accepted for publication by, medical journals that meet nationally recognized requirements for scientific manuscripts and that submit most of their published articles for review by experts who are not part of the editorial staff; (2) Peer-reviewed medical literature, including liter ...

THE SENTENCE A LETTER WOULD WRITE

An adverse benefit determination based on medical necessity is subject to external review by an independent review organization under Ohio's external review chapter.

WHY WE SELECTED IT

CO-50 is a medical-necessity denial; Chapter 3922 is Ohio's external-review chapter and 3922.01 is where its definitions, including adverse benefit determination, live. A reviewer should decide whether the operative request section is the better cite.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

We would particularly like your view on this one: 3922.01 is the definitions section of Ohio's external-review chapter. Should the operative section that creates the right to request external review be cited instead of, or in addition to, the definitions?

1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.20

Denial reason	CO-50 — Non-covered service because it is not deemed a medical necessity by the payer.
Payer type	Commercial (non-Medicare)
Official heading	Prohibition against unfair or deceptive acts
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.20

RETRIEVED TEXT

No person shall engage in this state in any trade practice which is defined in sections 3901.19 to 3901.23 of the Revised Code as, or determined pursuant to those sections to be, an unfair or deceptive act or practice in the business of insurance. This section applies to any person, as defined in section 3901.19 of the Revised Code, regardless of whether the person is licensed or required to be licensed by the superintendent of insurance.

THE SENTENCE A LETTER WOULD WRITE

No person shall engage in any trade practice defined as an unfair or deceptive act or practice in the business of insurance.

WHY WE SELECTED IT

The general prohibition a complaint to the Department of Insurance rests on; whether a bare CO-50 without disclosed criteria reaches it is a judgement for the reviewer, not the gate.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

We would particularly like your view on this one: does a medical-necessity denial that does not disclose the criteria it rests on plausibly reach the unfair-practices prohibition, or is citing 3901.20 overreach on these facts?

1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.381

Denial reason	OA-18 — Exact duplicate claim or service.
Payer type	Commercial (non-Medicare)
Official heading	Third-party payers processing claims for payment for health care services
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.381

RETRIEVED TEXT

... s adopted by the superintendent of insurance under section 3902.22 of the Revised Code, the third-party payer shall pay or deny the claim not later than thirty days after receipt of the claim. When a third-party payer denies a claim, the third-party payer shall notify the provider and the beneficiary. The notice shall state, with specificity, why the third-party payer denied the claim. (2)(a) Unless division (B)(3) of this section applies, when a provider or beneficiary has used the standard claim form, but the third-party payer determines that reasonable supporting documentation is needed to establish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than ...

THE SENTENCE A LETTER WOULD WRITE

A third-party payer shall pay or deny a claim not later than thirty days after receipt.

WHY WE SELECTED IT

A duplicate denial of a distinct service is a denial of a clean claim; the clock applies. Ohio statute does not define a duplicate claim.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Ohio Revised Code § 3901.381

Denial reason	PR-1 — Deductible amount.
Payer type	Commercial (non-Medicare)
Official heading	Third-party payers processing claims for payment for health care services
Retrieved from	codes.ohio.gov • https://codes.ohio.gov/ohio-revised-code/section-3901.381

RETRIEVED TEXT

... s adopted by the superintendent of insurance under section 3902.22 of the Revised Code, the third-party payer shall pay or deny the claim not later than thirty days after receipt of the claim. When a third-party payer denies a claim, the third-party payer shall notify the provider and the beneficiary. The notice shall state, with specificity, why the third-party payer denied the claim. (2)(a) Unless division (B)(3) of this section applies, when a provider or beneficiary has used the standard claim form, but the third-party payer determines that reasonable supporting documentation is needed to establish the third-party payer's responsibility to make payment, the third-party payer shall pay or deny the claim not later than ...

THE SENTENCE A LETTER WOULD WRITE

A third-party payer shall pay or deny a claim not later than thirty days after receipt.

WHY WE SELECTED IT

Only appealable when the deductible was misapplied; the statute supplies the reprocessing clock and nothing about deductible accumulation, which is plan-document territory.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

CMS Claims Processing Manual, Publication 100-04, Chapter 1, Section 80.3.2

Denial reason	CO-16 — Claim/service lacks information or has submission/billing error(s).
Payer type	Medicare
Official heading	Handling Incomplete or Invalid Claims
Retrieved from	cms.gov IOM · https://www.cms.gov/regulations-and-guidance/guidance/manuals/downloads/clm104c01.pdf

RETRIEVED TEXT

... ese data elements will affect whether or not an incomplete or invalid claim (hardcopy or electronic) will be "returned as unprocessable" by the A/B MAC (B) or "returned to provider" (RTP) by the A/B MAC (A). The contractor shall not deny claims and afford appeal rights for incomplete or invalid information as specified in this instruction. (See §80.3.1 for Definitions.) If a data element is required and it is not accurately entered in the appropriate field, the contractor returns the claim to the provider of service. • If a data element is required, or is conditional (a data element that is required when certain conditions exist) and the conditions of use apply) and is missing or not accurately entered in its appropriate ...

THE SENTENCE A LETTER WOULD WRITE

An incomplete or invalid claim is returned as unprocessable rather than denied.

WHY WE SELECTED IT

The MAC instruction that governs what a Medicare contractor does with a claim missing information.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

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☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

NCCI Policy Manual, Chapter 1, Section D

Denial reason	CO-97 — The benefit for this service is included in the payment or allowance for another service that has already been adjudicated.
Payer type	Medicare
Official heading	Evaluation & Management (E&M) Services
Retrieved from	cms.gov NCCI · https://www.cms.gov/files/document/01-chapter1-ncci-medicare-policy-manual-2026-final.pdf

RETRIEVED TEXT

... luded in the payment for the minor surgical procedure and shall not be reported separately as an E&M service. However, a significant and separately identifiable E&M service unrelated to the decision to perform the minor surgical procedure is separately reportable with modifier 25. The E&M service and minor surgical procedure do not require different diagnoses. If a minor surgical procedure is performed on a new patient, the same rules for reporting E&M services apply. The fact that the patient is “new” to the provider/supplier is not sufficient alone to justify reporting an E&M service on the same date of service as a minor surgical procedure. The NCCI program contains many, but not all, possible edits based on these pri ...

THE SENTENCE A LETTER WOULD WRITE

A significant and separately identifiable E&M service unrelated to the decision to perform a minor surgical procedure is separately reportable with modifier 25.

WHY WE SELECTED IT

The NCCI policy that CO-97 bundling edits implement; the separately-identifiable exception is stated there.

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1. Does this provision impose the obligation we describe?

☐ Yes ☐ No ☐ Partly

2. Is it appropriate to cite for this denial reason and this payer type?

☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

CMS Claims Processing Manual, Publication 100-04, Chapter 12, Section 30.6.6

Denial reason	CO-4 — The procedure code is inconsistent with the modifier used, or a required modifier is missing.
Payer type	Medicare
Official heading	Payment for Evaluation and Management Services Provided During
Retrieved from	cms.gov IOM • https://www.cms.gov/regulations-and-guidance/guidance/manuals/downloads/clm104c12.pdf

RETRIEVED TEXT

... vided on the day of a procedure with a global fee period if the physician indicates that the service is for a significant, separately identifiable E/M service that is above and beyond the usual pre- and post-operative work of the procedure. Different diagnoses are not required for reporting the E/M service on the same date as the procedure or other service. Modifier -25 is added to the E/M code on the claim. Both the medically necessary E/M service and the procedure must be appropriately and sufficiently documented by the physician or qualified nonphysician practitioner in the patient's medical record to support the claim for these services, even though the documentation is not required to be submitted with the claim. If ...

THE SENTENCE A LETTER WOULD WRITE

A significant, separately identifiable evaluation and management service on the same day as a procedure is reported with modifier 25.

WHY WE SELECTED IT

The MAC instruction on modifiers 24, 25 and 57 during the global period.

Our automated check confirms this provision exists, that its retrieved text contains every figure the sentence above asserts, and that it applies to this payer type in this state. It cannot confirm that it is the right provision to cite for this denial reason. That is the question on this sheet.

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☐ Yes ☐ No ☐ With caveats

3. If not — what should we cite instead, or should we cite nothing?

4. Wording you would require or forbid:

Anything else

If something about this approach strikes you as wrong in a way none of the questions above reaches, that is the most valuable thing you could tell us. We would rather hear it now than after the first letter goes out.

Reviewer name and date: _____